

1. Administrative & Study Identification

Full Study Title: A Phase III, Randomized, Open-Label, Multicenter Study of Lurbinectedin in Combination With Atezolizumab Compared With Atezolizumab as Maintenance Therapy in Participants With Extensive-Stage Small-Cell Lung Cancer (ES-SCLC) Following First-Line Induction Therapy With Carboplatin, Etoposide and Atezolizumab **Short Title/Acronym:** IMforte **Protocol Number:** G043104 **NCT Number:** NCT05091567 **Posting ID:** 255625691305017061 **Trial Status:** Active, Not Recruiting **Sponsor Name:** Roche (in collaboration with Genentech, Inc. and Jazz Pharmaceuticals) **Company:** Roche **Investigational Product:** Lurbinectedin (Zepzelca) + Atezolizumab (Tecentriq) **Phase of Development:** Phase III **Medical Monitor:** Sponsor Clinical Medical Director, Roche / Genentech

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To evaluate the efficacy of lurbinectedin in combination with atezolizumab compared with atezolizumab alone as first-line maintenance therapy in terms of Overall Survival (OS) and Progression-Free Survival (PFS) as assessed by an Independent Review Facility (IRF). **Secondary Objectives:** To assess Investigator-Assessed PFS, Confirmed Objective Response Rate (ORR) as determined by the IRF, and Confirmed ORR as determined by the Investigator, alongside Duration of Response (DoR), OS rates, and the overall safety and tolerability profile of the combination regimen.

2.2 Background & Rationale Extensive-stage small cell lung cancer (ES-SCLC) is an aggressive malignancy. While the addition of atezolizumab to platinum-etoposide chemotherapy established immunotherapy-based first-line treatment, relapse remains common, and long-term survival remains limited. Maintenance strategies that prolong survival are urgently needed. Lurbinectedin is a selective inhibitor of oncogenic transcription programs with established activity in relapsed SCLC. The IMforte study investigates whether the addition of lurbinectedin to atezolizumab maintenance significantly improves survival compared to standard-of-care atezolizumab monotherapy.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Active Comparator (Atezolizumab alone) **Blinding:** Open-label **Randomization:** 1:1 ratio (stratified by liver metastases at

induction baseline, receipt of prophylactic cranial irradiation, ECOG PS, and LDH at maintenance baseline)

3.2 Planned Enrollment & Duration Targeted Enrollment: 660 participants **Number of Sites:** 119 global locations **Estimated Study Start:** October 2021 **Estimated Primary Completion:** July 2024 (Data cutoff for primary analysis)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Induction & Maintenance Phases: 1. Adults (≥ 18 years) with histologically or cytologically confirmed extensive-stage small-cell lung cancer (ES-SCLC). 2. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 3. Adequate hematologic and end-organ function to receive induction and maintenance treatments. 4. Measurable disease, as defined by RECIST v1.1. 5. Negative HIV test and no evidence of active Hepatitis B or Hepatitis C at screening. 6. Treatment-free for at least 6 months since last chemotherapy/radiotherapy, among those treated (with curative intent) with prior chemo/radiotherapy for limited-stage SCLC. 7. *Maintenance Specific:* Completed 4 cycles of induction therapy with carboplatin, etoposide, and atezolizumab with an ongoing response or stable disease per RECIST v1.1. 8. *Maintenance Specific:* Toxicities attributed to prior induction anti-cancer therapy or prophylactic cranial irradiation (PCI) resolved to Grade ≤ 1 .

4.2 Exclusion Criteria Induction & Maintenance Phases: 1. Presence or history of CNS metastases. 2. Prior systemic therapy for ES-SCLC. 3. Active or history of autoimmune disease or deficiency. 4. History of malignancies other than SCLC within 5 years prior to enrollment. 5. Prior treatment with CD137 agonists or immune checkpoint blockade therapies (including anti-CTLA-4, anti-PD-1, and anti-PD-L1 therapeutic antibodies), lurbinectedin, or trabectedin. 6. History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, or idiopathic pneumonitis, or evidence of active pneumonitis on screening chest computed tomography (CT) scan. 7. Treatment with investigational therapy within 28 days prior to enrollment. 8. *Maintenance Specific:* Receiving consolidative chest radiation. 9. *Maintenance Specific:* Severe infection within 2 weeks prior to randomization into the maintenance phase, or treatment with therapeutic oral or IV antibiotics at the time of randomization.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Induction Phase:** Atezolizumab + Carboplatin + Etoposide (trade names: Etopophos, Vepesid, Toposar; ATC Code: L01CB01) every 21 days for 4 cycles. * **Combination Arm (Maintenance):** Lurbinectedin 3.2 mg/m^2 intravenously (IV) every 21 days (q3w) with G-CSF prophylaxis +

Atezolizumab \$1200\$ \$mg\$ IV every 21 days (q3w). * **Control Arm (Maintenance):** Atezolizumab \$1200\$ \$mg\$ IV every 21 days (q3w). **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Cycles repeated every 21 days until disease progression, unacceptable toxicity, or withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: G-CSF prophylaxis (required for patients in the lurbinectedin arm). Standard supportive care for chemotherapy-induced side effects. **Prohibited:** Crossover between study arms was strictly prohibited. Concurrent use of other investigational anti-cancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Up to Day -28	Informed Consent, Medical History, Tumor Assessment, Labs
2	Cycles 1-4	Atezolizumab + Carboplatin + Etoposide (every 21 days)
3	Day 1	Randomization, First Maintenance Dose, Baseline Imaging
4	Every 21 Days	Treatment administration, Safety Labs, AE/SAE Assessment
5	End of Treatment Visit	Progression/Toxicity, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. **IRF-Assessed Progression-Free Survival (PFS):** Defined as the time from randomization to the date of first documented disease progression (as assessed by the IRF according to RECIST v1.1), or death, whichever occurs first. 2. **Overall Survival (OS):** Defined as the time from randomization to the date of death from any cause.

7.2 Secondary Endpoints 1. Investigator-Assessed Progression-Free Survival (PFS). 2. Confirmed Objective Response Rate (ORR) as Determined by the IRF. 3. Confirmed Objective Response Rate (ORR) as Determined by the Investigator. 4. Duration of Response (DoR), PFS rates at 6 and 12 months, and OS rates at 12 months. 5. Incidence and severity of Adverse Events (AEs) and time to confirmed deterioration.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard continuous AE collection every 21-day cycle, graded via CTCAE. Specific monitoring for myelosuppression, hepatotoxicity, extravasation, and immune-mediated adverse events. **Serious Adverse Events (SAEs):** Mandatory reporting within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** External Independent Data Monitoring Committee (iDMC) to periodically assess safety and efficacy data.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy (OS, PFS) analyses; Safety Set for all patients who received at least one dose of study treatment. **Sample Size Justification:** Target enrollment of 660 provides sufficient power for the detection of statistically significant improvements in OS and IRF-assessed PFS ($\alpha = 0.05$ split between endpoints). **Handling of Missing Data:** Patients without documented progression or death were censored at the date of their last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) guidelines and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring visits by sponsor-designated clinical research associates (CRAs) to ensure protocol compliance and data integrity. **Audit Trail:** Robust eCRF locking, source data verification (SDV), and data clarification forms managed through centralized electronic data capture (EDC) systems.

11. Study Identifiers & Governance

Primary ID: G043104 **Registry Number:** NCT05091567 **Posting ID:** 255625691305017061 **Sponsor/Lead Organization:** Roche **Study Title:** A Phase III, Open-Label Study of Maintenance Lurbinectedin in Combination With Atezolizumab Compared With Atezolizumab in Participants With Extensive-Stage Small-Cell Lung Cancer **Official Regulatory Title:** A Phase III, Randomized, Open-Label, Multicenter Study of Lurbinectedin in Combination With Atezolizumab Compared With Atezolizumab as Maintenance Therapy in Participants With Extensive-Stage Small-Cell Lung Cancer (ES-SCLC) Following First-Line Induction Therapy With Carboplatin, Etoposide and Atezolizumab

12. Clinical Framework

Disease Area: Small-Cell Lung Cancer **Preferred UMLS Name:** Small Cell Lung Carcinoma **Diagnosis Threshold:** Histologically or cytologically confirmed ES-SCLC **Terminology Codes:** MeSH: D008173 | SNOMED CT: 254639001 (Hierarchy: 254636003 | Lung carcinoma | Lung carcinoma (disorder)) **Medical Methodology:** 1L Maintenance Therapy following Induction **Optimization Target:** Prolongation of Overall Survival (OS) and Progression-Free Survival (PFS)

Management Pathway: 4 cycles of induction (Platinum/Etoposide + Atezolizumab) followed by 1L Maintenance (Atezolizumab + Lurbinectedin) **Education Protocol (HCP):** Investigator meeting and protocol training for site staff regarding AE management (e.g., G-CSF prophylaxis guidelines) **Education Protocol (Patient):** Informed consent, safety counseling regarding immune-mediated and myelosuppressive side effects **Quality Audit Mechanism:** Independent Review Facility (IRF) for centralized, unbiased tumor response evaluation

Total Duration: Median follow-up of 15.0 months at data cutoff; patients followed until death or study withdrawal **Onsite Visit Cadence:** Every 21 days (Q3W) corresponding to the treatment cycles **Remote Monitoring Frequency:** Routine data reviews per the clinical monitoring plan **Checklist Review Interval:** Pre-dose clinical and laboratory assessments every 21 days (Q3W)

Role	Name	Signature	Date	_____	_____	_____	_____
Principal Investigator	[Insert Name]	_____	[DD-MMM-YYYY]	_____	_____	_____	_____
Clinical Operations Lead	[Insert Name]	_____	[DD-MMM-YYYY]	_____	_____	_____	_____
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]	_____	_____	_____	_____